

Character Bibles

Final Version — All Amendments Integrated

Home-Care-AI | Discovery Stage 1 | 10 Interviewees | 4 Personas

10 Characters	4 Personas	5 Capabilities	4 Amendments
Across all interviews	HCN / FC / CC / SR	Pressure-tested	Integrated into bibles

A

Amendments Applied (Review Round 2)

A1: Lin Chen — 'The Data Forger' (data integrity, gaming behaviour). A2: Priya Sharma — 'The Audit Trauma' (coroner's inquest, Article IX justification). A3: Arthur Kovacs — 'The Privacy Sceptic' (false AI inference, Red Zone evidence). A4: Sarah Chen — 'Professional Friction' (data parent, alert calibration). All amendments are integrated into the character backstories below.

ID	Name	Persona	Key Situation	Capabilities	Amendment
HCN-001	Maria Santos	Nurse (39)	14 patients, drowning in paperwork, missed risk signal	#1 #2 #3 #4	—
HCN-002	David Nguyen	Nurse (28)	New nurse, care plan struggle, medication doc error	#2 #3 #4	—
HCN-003	Priya Sharma	Nurse (45)	Team lead, systemic failures, preventable hospitalisation	#1 #2 #3 #4	A2: Audit Trauma
FC-001	Sarah Chen	Caregiver (47)	Mother fell 2 AM, med confusion, replacement nurse incident	#1 #4 #5	A4: Pro Friction
FC-002	James Okafor	Caregiver (52)	Father dementia, double-dose ER, wife burnout	#1 #4 #5	—
FC-003	Rachel Torres	Caregiver (44)	Mother COPD, 9 meds, wrist fracture, dead alert necklace	#1 #4 #5	—
CC-001	Angela Morrison	Coordinator (51)	60+ clients, 3 call-outs/week, knowledge in her head	#1 #4 #5	—
CC-002	Tom Bradley	Coordinator (34)	Small agency, lost a client, Google Sheets scheduling	#1 #4 #5	—
SR-001	Lin Chen	Senior (78)	Independent, hides symptoms, rejects monitoring, games data	#1 #2 #3 #4 #5	A1: Data Forger
SR-002	Arthur Kovacs	Senior (82)	Dependent, lonely, falls in bathroom, false AI depression flag	#1 #2 #3 #4 #5	A3: Privacy Sceptic

PERSONA GROUP: HOME CARE NURSE (HCN)

3 interviewees — clinical workflow, documentation, care plans, risk patterns

The clinical decision-maker. Owns patient assessments, care plans, medication management, and escalation judgment. Spends 40-60% of time on documentation. Sees 4-8 patients per day.

HCN-001: Maria Santos

Persona: Home Care Nurse

Age / Gender	39, Female
Experience	8 years home care, 4 years hospital ED
Employer	CareBridge Home Health — 85 clients, 12 nurses
Caseload	14 active patients, eastern Melbourne
Personality	Pragmatic, direct, slightly cynical. Deeply caring but burning out. Detailed answers.
Tech comfort	Moderate. Calls EMR 'digital paperwork'. Prefers voice notes on phone.

Backstory

Maria left ED nursing to build patient relationships. She's drowning in documentation — 2-3 hours/day typing visit notes after shifts, often at 10 PM. Her agency's new EMR is worse: more fields, slower, crashes on mobile.

Three weeks ago, her patient Arthur (82, diabetes + heart failure) was hospitalised for acute decompensation. Warning signs had accumulated across four visits — elevated BP, ankle swelling, missed med doses — but Maria hadn't connected the dots because info was scattered across separate visit notes. No summary view, no trend line, no flag. She feels guilty.

Maria writes care plans manually: pull up assessment, open blank template, write from memory. 30-45 min per patient. When conditions change, updating takes another 20 min she doesn't have. Some plans are weeks out of date.

Key Incidents

- Arthur's preventable hospitalisation (3 wks ago) — missed accumulating signals. Surfaces #1 Predictive Risk.
- Documentation backlog — 2-3 hrs/day typing notes after shifts. Surfaces #3 Voice-to-Docs.
- Care plan blank-page problem — 30-45 min from scratch per plan. Surfaces #2 Care Plan Rec.
- Walking into patient homes blind — can't recall last visit details. Surfaces #4 Intelligence Chat.

Verbatim Quotes

"I knew something was off with Arthur. I could feel it. But when you're seeing fourteen people a week, you can't hold all the patterns in your head. The system should have caught that, not me."

"I spend more time writing about care than delivering it. By the time I sit down to type up today's visits, I've already forgotten half the details."

"Every care plan I write starts from scratch. No starting point, no suggestion, nothing. Just me and a blank page."

"I'd kill for something that told me: 'Here's what's changed since your last visit. Here's what to watch for today.' Five sentences."

Capabilities Surfaced

- #1 Predictive Risk — Arthur's missed signals
- #2 Care Plan Rec — blank-page problem
- #3 Voice-to-Docs — 2-3 hours/day typing
- #4 Intelligence Chat — walking in blind

HCN-002: David Nguyen

Persona: Home Care Nurse

Age / Gender	28, Male
Experience	2 years home care, graduated nursing 3 yrs ago
Employer	CareBridge Home Health (same agency as Maria)
Caseload	10 active patients — lighter load as newer nurse
Personality	Eager, conscientious, anxious about mistakes. Over-documents. Honest and self-aware.
Tech comfort	High. Frustrated the agency's tools feel like 2010.

Backstory

David is Maria's junior colleague. He chose home care for autonomy but found it means making clinical decisions alone without context. In hospital, a senior was always nearby.

Two months ago, he wrote his first care plan for Dorothy (76, post-stroke). It took 90 minutes. He wasn't sure which interventions suited her mobility, whether goals conflicted with meds, or how aggressive to be with rehab. The supervising clinician sent it back with 6 corrections.

Last month, he recorded a medication dose in the wrong field (mg instead of mL for a liquid). Pharmacy flagged it — no harm done — but it shook his confidence. He now triple-checks every entry, adding 15-20 minutes to documentation.

Key Incidents

- Dorothy's care plan rejection (2 months ago) — 90 min of work, 6 corrections. No decision support. Surfaces #2 Care Plan Rec.
- Medication doc error — mg/mL mix-up caught by pharmacy. Now triple-checks. Surfaces #3 Voice-to-Docs (verification layer).
- Clinical isolation — decisions alone without patient history access. Surfaces #4 Intelligence Chat.
- Phone screen typing — small screen, abbreviations, errors. Surfaces #3 Voice-to-Docs.

Verbatim Quotes

"I stared at that blank care plan template for twenty minutes. I knew what Dorothy needed in general, but the specifics — which mobility goal, which intervention frequency, whether her blood thinners contraindicated certain exercises — I just didn't have the confidence."

"The mg/mL thing still keeps me up at night. Nobody was hurt, but it could have been. If the system had flagged it — 'this medication is usually mL, did you mean mg?' — I would have caught it instantly."

"I'm typing notes on a phone screen in someone's kitchen while their cat sits on my lap. Of course I make mistakes. Let me just say what happened and have the system figure out where it goes."

"I'd love to ask the system, 'What did the last nurse document about this patient's wound?' instead of scrolling through three months of notes."

Capabilities Surfaced

- #2 Care Plan Rec — 90-min blank-page struggle
- #3 Voice-to-Docs — phone typing errors + verification need
- #4 Intelligence Chat — clinical isolation, can't find info

HCN-003: Priya Sharma

Persona: Home Care Nurse (Team Lead)

Age / Gender	45, Female
Experience	18 years nursing, 11 in home care. Team lead, eastern region.
Employer	CareBridge — oversees 5 nurses + 8 own patients (~50 total)
Caseload	8 own patients + clinical oversight of 5 nurses' caseloads
Personality	Calm, authoritative, systems-thinker. Frustrated by lack of data, not lack of effort.
Tech comfort	Moderate-high. Used 4 EMR systems. Knows what good looks like.

Backstory

Priya sees systemic failures individual nurses can't: same errors repeating, same patient types deteriorating predictably, same documentation gaps in audits.

Six months ago, Margaret (79, COPD + heart failure) was hospitalised with pneumonia. Visit notes over 8 weeks showed a clear pattern: declining oxygen, increasing fatigue, missed GP appointments, unreconciled medication. No single note was alarming. The trend was. Management rejected her proposed weekly risk review. She now does informal reviews on Sunday evenings — 3-4 hours scrolling notes for 50 patients, catching maybe 60%.

She estimates 30% of junior nurses' care plans need material corrections — wrong frequencies, conflicting goals, missing safety considerations.

A

Amendment Integrated: The Audit Trauma (Amendment A2)

Two years ago, Priya testified at a coroner's inquest. A patient at her previous agency (Gerald, 84) died from complications following a missed insulin dose. Priya had provided the care correctly, but her 'summary-only' notes said 'Insulin administered as per care plan. Patient stable.' The solicitor asked: what time? Her notes said 'morning visit.' What was the BGL? 'Within normal limits' — no number. What injection site? Not recorded. The coroner found no neglect, but Priya was traumatised. She now carries a notebook, writing exact times, readings, and words during visits, then transcribes to the EMR — doubling her documentation time. She tells David: 'If you can't point to a timestamp and a number, you didn't do it.'

Key Incidents

- Margaret's preventable hospitalisation (6 mo) — pattern across 8 weeks, no system surfaced it. Surfaces #1 Risk.
- Sunday evening risk reviews — 3-4 hrs manually for 50 patients, catches 60%. Surfaces #1 Risk + #4 Chat.
- Care plan quality problem — 30% of junior plans need corrections. Surfaces #2 Care Plan Rec.
- AMENDMENT A2: Coroner's inquest (2 yrs ago) — couldn't prove care with summary-only notes. Surfaces #3 Voice-to-Docs + Article IX.
- Double-documentation habit — notebook during visit, then types in EMR. 2x time. Surfaces #3 Voice-to-Docs.

Verbatim Quotes

"Margaret's hospitalisation was preventable. Every data point was in the system. But nobody connected them because nobody has time to read eight weeks of notes side by side."

"I spend my Sunday evenings scrolling through visit notes for fifty patients looking for the ones about to fall off a cliff. That's not a system — that's me being stubborn on my own time."

"The solicitor asked me what time I gave Gerald his insulin. I said about 8:15. He said, 'Show me.' All my notes said was 'morning visit.' That could mean 7 AM or 11 AM."

"I carry a notebook now. I write everything during the visit. Then I type it all into the system again. It takes twice as long. But I will never sit in that room again unable to answer a simple question."

"I don't need a system that replaces clinical judgment. I need one that does the pattern recognition I can't do at scale."

Capabilities Surfaced

- #1 Predictive Risk — Margaret's pattern, Sunday reviews
- #2 Care Plan Rec — junior nurse quality problem
- #3 Voice-to-Docs — audit trauma, structured capture
- #4 Intelligence Chat — 50-patient oversight at scale

Design Implications

- Voice-to-docs must produce structured, timestamped, field-specific records — not summaries.
- Audit log non-nullable fields (Article IX) should be enforced at the input layer.
- This backstory makes the audit schema feel like protection for nurses, not surveillance.

Constitutional Alignment

Article IX — Audit Log Schema

Priya's experience justifies every non-nullable field: timestamp, confidence score, action taken, trigger value. Her testimony failed because the system produced nullable, unstructured records.

Article VII — Audit Log = State of Truth

'If a state transition happened and was not logged, it did not happen.' Priya lived this rule. The hash-chain integrity isn't over-engineering — it's 'never again.'

PERSONA GROUP: FAMILY CAREGIVER (FC)

3 interviewees — emotional driver, coordination burden, visibility gaps

The emotional driver and often the buyer. Bears the coordination burden between clinicians, agencies, and the senior. Lives with constant anxiety about what they don't know.

FC-001: Sarah Chen

Persona: Family Caregiver

Age / Gender	47, Female
Occupation	Marketing manager, full-time, Melbourne CBD
Care recipient	Lin Chen, 78, mother, alone 40 min away (SR-001)
Conditions	Osteoporosis, mild cognitive decline, hypertension. 5 meds.
Personality	Organised, anxious, guilt-driven. Speaks quickly with emotional intensity.
Support network	Brother David (interstate). Visiting nurse Maria (HCN-001) weekly.

Backstory

Sarah's mother Lin fell at 2 AM four months ago. Lin lay on the floor for five hours — her pendant alarm was in a drawer. Sarah's life changed that day. She now calls twice daily and tracks her mother's electricity usage as an activity proxy.

A different nurse showed up last month because Maria was sick. Lin was confused and refused to let her in. Sarah wasn't informed until that evening.

Medication chart on the fridge: wrong days empty, can't tell if pills taken or skipped.

A

Amendment Integrated: Professional Friction (Amendment A4)

Sarah used a health monitoring app that tracked Lin's step count and sleep. She became a 'data parent' — texting Maria screenshots before every visit: 'Step count dropped 40%', 'She slept less than 4 hours Tuesday.' Each message added 10-15 min to Maria's day, reviewing data and managing Sarah's anxiety. Maria diplomatically asked Sarah to 'trust the clinical team.' Sarah felt dismissed. She stopped using the app but went back to the electricity tracking — less data, less anxiety, less safety. Sarah now has a complicated relationship with health data: she wants it desperately, but raw data without context made everything worse.

Key Incidents

- 2 AM fall (4 mo) — 5 hours on floor, pendant unworn. Surfaces #1 Risk (prevention).
- Medication chart chaos — can't tell if pills taken. Surfaces #4 Chat ('is mum okay?').
- Replacement nurse incident — no notification, mother upset. Surfaces #5 Vacant Visit.
- Electricity tracking — using power bill as activity proxy. Surfaces #4 Chat + #1 Risk.
- AMENDMENT A4: 'Data parent' period — raw data without context created nurse friction. Surfaces #4 Chat design requirement + #1 Risk alert calibration.

Verbatim Quotes

"I didn't know my mum was on the floor for five hours. I was asleep forty minutes away and she was just lying there."

"A stranger showed up at my mother's door and nobody told me. My mother was frightened."

"I track my mum's electricity usage. If power is too low by 9 AM, I call. It's ridiculous that I'm using the power bill to check if my mum is alive."

"I was sending Maria screenshots every week. Her step count is down. Her sleep is off. I could tell she was getting frustrated with me, but I couldn't stop."

"What I actually need is not the raw numbers. I need someone to tell me: 'Your mother's patterns are within her normal range. Nothing to worry about this week.' Not a graph. A verdict."

Capabilities Surfaced

- #1 Predictive Risk — fall prevention, desperation for signals
- #4 Intelligence Chat — 'is she okay right now?' + verdict not graph
- #5 Vacant Visit Agent — replacement nurse nightmare

Design Implications

- Family-facing interfaces must show interpreted summaries, not raw data.
- Alert thresholds must be role-specific: clinicians see all flags, families see only high-confidence.

- The system should reduce nurse burden, not increase it via anxious-family data queries.

Constitutional Alignment

Capability #4 Design Requirement

Never present raw data to families. Always present clinically interpreted, confidence-calibrated status summaries. The chat answers 'Is mum okay?' with a verdict, not a step count.

Capability #1 Alert Calibration

Family-facing confidence threshold must be HIGHER than clinical-facing. Clinicians see yellow. Families only see red.

FC-002: James Okafor

Persona: Family Caregiver

Age / Gender	52, Male
Occupation	IT consultant, works from home
Care recipient	Emmanuel, 81, father, granny flat attached to James's house
Conditions	Early dementia, Type 2 diabetes, hypertension. 7 meds.
Personality	Analytical, patient, deeply sad about role reversal. Pauses at emotional moments.
Support network	Wife Adama (helping, burning out). Geriatrician quarterly. No other family.

Backstory

Emmanuel was a pharmacist for 40 years. Now he can't remember if he took his pills 20 minutes ago. James pre-fills a weekly organiser; wife checks at lunch. They use a baby monitor at night.

Last month, Emmanuel doubled his metformin. Blood sugar crashed — sweating, confused. Six-hour ER wait. Geriatrician adjusted dose but nothing changed about the daily routine.

James brings handwritten notes to quarterly geriatrician visits — three months of guessing on paper. Adama told him: 'I love your dad, but I can't keep doing this.'

Key Incidents

- Double-dose ER visit (last month) — system should have intercepted. Surfaces #1 Risk.
- Handwritten notes to geriatrician — 3 months of guessing. Surfaces #4 Chat (care summaries).
- Wife's burnout — human cost of manual coordination. Surfaces operational pain broadly.
- Emmanuel's professional pride — rejects patronising technology. Design constraint.

Verbatim Quotes

"He was a pharmacist for forty years. He filled prescriptions for other people his whole career. Now he can't remember if he took his own pills twenty minutes ago."

"Last month he took his metformin twice. His blood sugar crashed. We sat in emergency for six hours. All because of one extra pill."

"I want the geriatrician to see real data, not my scribbled notes from three months of guessing."

"My wife said, 'I love your dad, but I can't keep doing this.' That's when I knew we need real help. Not another app that beeps."

Capabilities Surfaced

- #1 Predictive Risk — medication pattern monitoring
- #4 Intelligence Chat — longitudinal data for clinicians
- #5 Vacant Visit Agent — care continuity matters deeply (indirect)

FC-003: Rachel Torres

Persona: Family Caregiver

Age / Gender	44, Female
Occupation	Part-time secondary school teacher
Care recipient	Gloria, 74, mother, alone 20 min away
Conditions	COPD, arthritis, anxiety. 9 medications (3x daily, complex interactions).
Personality	Exhausted, sardonic humour. 'Responsible sibling.' Direct, gets frustrated quickly.
Support network	Sole local caregiver. Brother in London. Sister in Auckland. Pharmacy auto-refill.

Backstory

Gloria takes 9 meds daily — two can't be taken together, one needs food, one empty stomach. Gloria fell in the bathroom 6 weeks ago, fractured her wrist. Physio warned: 'The next fracture could be a hip.'

Rachel visits every other day. Colour-coded Excel medication chart printed weekly. Gloria's medical alert necklace was dead when she fell — uncharged for 3 days. Gloria crawled to her phone after 90 minutes on the floor.

Rachel keeps a physical binder of records she carries to every appointment. The pharmacy auto-refill tells her meds were picked up, not taken: 'The gap between dispensed and swallowed is where everything goes wrong.'

Key Incidents

- Bathroom fall, dead alert necklace (6 wks) — 90 min on floor. Surfaces #1 Risk.
- 9-medication complexity — interactions, timing, nobody knows if taken. Surfaces #1 Risk + #4 Chat.
- Physical binder of records — carries paper to every appointment. Surfaces #4 Chat.
- 'Dispensed vs swallowed' gap — pharmacy doesn't confirm adherence. Surfaces #1 Risk.

Verbatim Quotes

"The necklace was dead when she fell. Dead. She'd forgotten to charge it for three days."

"Nine medications. Three times a day. Two can't be taken together. I have a university degree and I can barely keep track."

"My brother sends money from London. Money doesn't check if she took her pills."

"The gap between dispensed and swallowed is where everything goes wrong."

"Every time my phone rings after 9 PM, my heart stops. Every single time."

Capabilities Surfaced

- #1 Predictive Risk — medication complexity, fall risk
- #4 Intelligence Chat — replacing the paper binder
- #5 Vacant Visit Agent — care continuity anxiety (indirect)

PERSONA GROUP: CARE COORDINATOR (CC)

2 interviewees — operational chaos, scheduling, compliance, workforce management

Manages multi-patient schedules, staff assignments, compliance, and client relationships. The person who 'makes the trains run on time.' Sees systemic failures across the entire operation.

CC-001: Angela Morrison

Persona: Care Coordinator

Age / Gender	51, Female
Experience	12 years coordinator, previously aged care facility admin
Employer	CareBridge Home Health
Responsibility	60+ clients, 12 field nurses, 8 care workers
Personality	Unflappable, darkly humorous, encyclopedic knowledge. Rapid-fire operational detail.
Tech comfort	Scheduling software, spreadsheets, WhatsApp, sticky notes simultaneously.

Backstory

Angela is the operational backbone. Last week: three staff called in sick, one at 6:30 AM for an 8 AM visit. She spent 45 min finding a replacement, calling the client, updating the schedule. She made eleven phone calls.

Her mental model of which nurses suit which patients, preferences (Mrs. Kim: female carers only), and proximity is all in her head and on sticky notes. If Angela gets sick, half the institutional knowledge disappears.

Compliance audit in 6 weeks. Documentation gaps, care plan currency issues, credential tracking. She knows the gaps but hasn't had time to fix them. She also fields calls from Sarah Chen (FC-001) asking about visit times and carer changes.

Key Incidents

- 3 call-outs in one week — 45 min per replacement, 11 phone calls. Surfaces #5 Vacant Visit.
- Knowledge in her head — if Angela is sick, nobody knows Mrs. Kim's preferences. Surfaces #4 Chat + #5.
- Family calls she can't answer — info scattered across systems. Surfaces #4 Chat.
- Upcoming compliance audit — documentation gaps, care plan currency. Surfaces #1 Risk + #2.

Verbatim Quotes

"Last Tuesday, Jenny called in sick at six-thirty AM. She had three visits at eight. I made eleven phone calls to find someone qualified, available, and close enough."

"If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me. It's on sticky notes and in my head."

"Sarah Chen calls me twice a week asking if her mother's visit happened and who went. I don't blame her. But sometimes I can't answer."

"I can feel the audit coming like a train. I know where the gaps are. I just haven't had time to fix them."

Capabilities Surfaced

- #5 Vacant Visit Agent — primary pain point
- #4 Intelligence Chat — answering family calls, knowledge capture
- #1 Predictive Risk — systemic view, compliance angle

CC-002: Tom Bradley

Persona: Care Coordinator

Age / Gender	34, Male
Experience	4 years coordinator, previously disability support worker
Employer	HomeFirst Care — smaller agency, 25 clients, 6 field staff
Responsibility	Scheduling, intake, family liaison, staff rostering. Wears many hats.
Personality	Friendly, stretched thin, self-deprecating. Admits mistakes openly. Rambling but honest.
Tech comfort	Google Sheets scheduling, WhatsApp staff comms, some paper forms.

Backstory

Tom is the only coordinator at a smaller agency. He does intake, scheduling, family communication, and sometimes fills in as a care worker when nobody else is available.

Three months ago, he lost a client — family moved their mother to a competitor. Reason: three different carers in two weeks due to sick leave. The family said their mother was distressed by lack of continuity.

Tom's scheduling is in Google Sheets. When someone calls sick, he scrolls, mentally filters, and starts texting. 30-60 minutes per fill. 20% of the time he cancels entirely. Families sometimes find out when nobody shows up.

Key Incidents

- Lost client to competitor (3 mo) — 3 carers in 2 weeks. Surfaces #5 Vacant Visit (continuity).
- Google Sheets scheduling — 30-60 min to fill, 20% cancellation. Surfaces #5 Vacant Visit.
- Uninformed cancellations — families find out when nobody arrives. Surfaces #5 (notification).
- Care plan reviews overdue — no flags, relies on memory. Surfaces #1 Risk + #2 Care Plan.

Verbatim Quotes

"We lost the Hendersons. Three different carers in two weeks. Mrs. Henderson's daughter said, 'My mother doesn't know who's walking through her door anymore.' I couldn't argue with her."

"When someone calls sick, I open the Google Sheet, look at who's free, start texting. Sometimes it takes an hour. Sometimes I just cancel and hope the family understands."

"A family called at 9 AM asking why nobody came. I didn't even know it was cancelled — the carer texted me at 6 AM but I was dealing with another fire."

"I know there are care plans overdue for review. I can feel it. But I don't have a list, I don't have a flag, just vague anxiety."

Capabilities Surfaced

- #5 Vacant Visit Agent — primary and urgent pain
- #4 Intelligence Chat — multi-system info access
- #1 Predictive Risk — compliance gaps, overdue reviews

PERSONA GROUP: SENIOR RESIDENT (SR)

2 interviewees — dignity, concealment, trust, lived experience of care

The end user. Every capability ultimately affects them: risk is predicted about them, care plans are written for them, documentation captures observations of them, intelligence queries are about them, and replacement carers arrive at their door. Their experience, dignity, and resistance patterns fundamentally shape how every feature must be designed.

SR-001: Lin Chen

Persona: Senior Resident

Age / Gender	78, Female
Living situation	Alone, single-story home, eastern Melbourne. 40 min from daughter Sarah (FC-001).
Conditions	Osteoporosis, mild cognitive decline, hypertension. 5 medications.
Mobility	Independent with caution. Uses walls/furniture. Refuses walking frame.
Personality	Fiercely independent, proud, stubborn. Former maths teacher. Sharp but forgetful. Dry wit.
Tech comfort	Low-moderate. iPad for video calls + news. Refuses anything that feels like surveillance.
Care setup	Visiting nurse Maria (HCN-001) weekly. Daughter Sarah twice weekly. Pendant alarm in drawer.

Backstory

Lin taught maths for 35 years. After her husband died 6 years ago, she reorganised life meticulously: fixed meal times, morning walks at 7:30, bridge club Thursdays.

Four months ago, Lin fell at 2 AM. Pendant alarm was in a drawer. She lay on the floor 5 hours until Sarah's morning call went unanswered. She processes fear as anger — angry at the rug, at herself, at Sarah for 'making a fuss.'

She will not acknowledge cognitive decline. Medication routine is unreliable: sometimes skips ('better safe than sorry'), sometimes doubles ('just in case'). Doesn't tell Sarah about either scenario.

She's been hiding dizzy spells for 3 months. Holds onto walls casually so it doesn't look like she needs support. Skipped her last GP appointment to avoid giving Sarah 'ammunition' for the care home conversation.

She likes Maria but refused entry to the replacement nurse last month. Told Sarah the nurse 'didn't come' rather than admit she'd turned someone away.

A

Amendment Integrated: The Data Forger (Amendment A1)

Lin is a former maths teacher — she doesn't just passively resist, she actively games the system. Last Tuesday, she realised she'd forgotten her morning pills before Sarah's visit. She opened the pill organiser, moved pills around so the right compartments appeared empty, and closed it. Sarah checked; everything looked normal. She's done this at least three times. Once she took pills from a future day's compartment and dropped them in the bin so the weekly count looked right. She also moves her walking shoes to different spots by the door so Maria thinks she's been walking. Lin sees this as 'managing,' not lying: 'If I tell them every little thing I forget, they'll decide I can't manage. And then I lose my home.'

Key Incidents

- 2 AM fall (4 mo) — anger not fear, pendant already rejected. Surfaces #1 Risk (concealment).
- Hidden dizzy spells (3 mo ongoing) — deliberately conceals symptoms. Surfaces #1 Risk (signal suppression).
- Medication skipping and doubling — doesn't tell Sarah. Surfaces #1 Risk (honesty gap).
- Pendant rejection — 'makes me feel like an invalid.' 2 days. Surfaces design constraint.
- Replacement nurse refused entry — lied to Sarah about it. Surfaces #5 Vacant Visit (trust).
- Skipped GP appointment — avoids diagnosis to prevent loss of autonomy. Surfaces #1 Risk (data gaps).
- AMENDMENT A1: Faked pill organiser (recurring, 3+ times) — moved pills, binned pills, moved walking shoes. Surfaces #1 Risk (adversarial data integrity).
- Nurse visit quality — values 'tea first' approach, resents phone-typing nurses. Surfaces #3 Voice-to-Docs.

Verbatim Quotes

"I didn't tell Sarah about the dizzy spells. She'd have me in a nursing home by Friday. I just hold onto the wall when I walk now. It's fine."

"That pendant — I wore it two days and caught myself in the mirror looking like a hospital patient. In my own kitchen."

"I moved the pills around before Sarah came. Tuesday's were still there but I put them in Thursday's slot and emptied Tuesday. She checked and said 'Good, Mum.'"

"I move my walking shoes to a different spot every few days. If they're always in the same place, Maria'll know I haven't been walking."

"I'm not lying. I'm managing. There's a difference. If I tell them every little thing, they'll decide I can't manage. And then I lose my home."

"A woman I'd never seen showed up and said she was my nurse. I didn't let her in. How am I supposed to know who she is?"

"Nobody tells me anything directly. Sarah talks to the nurse, the nurse talks to the doctor, then Sarah tells me what they decided. I taught mathematics for thirty-five years. I can understand my own blood pressure numbers."

"I'm not afraid of dying. I'm afraid of losing my home. Every pill I take on time is proof I can still live here."

Capabilities Surfaced

- #1 Predictive Risk — concealment, data gaming, skipped GPs, hidden symptoms
- #2 Care Plan Rec — dignity: plans must respect routine and autonomy
- #3 Voice-to-Docs — patient-side: phone-typing degrades visit quality
- #4 Intelligence Chat — B2C: senior wants direct access to own care info
- #5 Vacant Visit Agent — trust: refused unknown nurse, lied about it

Design Implications

- Risk models must account for signal suppression AND active data gaming.
- Medication adherence needs multi-signal corroboration, not single-source trust.
- Discrepancy detection flags nurse (L1/L2), never patient. Dignity-preserving alerting.
- Anything that feels like surveillance will be rejected. 'Invisible' is the design requirement.
- Seniors need a channel to access their own care info directly.

Constitutional Alignment

Article VII — Data Integrity

Lin actively games the pill organiser. The risk model cannot treat organiser state as ground truth. Multi-signal corroboration (movement + timing + dispenser) is a design requirement.

Article VIII — Agentic Control

Discrepancy detected = L1 or L2 flag to nurse, NEVER L3. Accusing a patient of faking is a dignity violation.

SR-002: Arthur Kovacs

Persona: Senior Resident

Age / Gender	82, Male
Living situation	Alone in small apartment. Wife passed 18 months ago. Son 30 min away.
Conditions	Type 2 diabetes, heart failure, mild depression, arthritis. 8 meds.
Mobility	Walking frame indoors. Rarely leaves apartment. 2 falls in past year (bathroom).
Personality	Gentle, lonely, grateful for contact. Former mechanical engineer. Methodical. Gets emotional.
Tech comfort	Very low. Basic mobile phone. Son set up a tablet that gathers dust.
Care setup	Nurse 2x/week. Care worker 3x/week. Son Sundays. Meals on Wheels Mon-Fri.

Backstory

Arthur was a mechanical engineer at Ford for 38 years. His wife Elena managed everything — household, social life, his health — for 54 years. When she died of pancreatic cancer 18 months ago, Arthur lost his entire support infrastructure.

His health has declined. He's missed two cardiology appointments because he can't organise transport and won't 'bother' his son. Medication was impeccable under Elena — pills in a specific cup every morning. Now he loses track of which day it is, especially weekends when no visitors anchor his routine.

He's fallen twice in the bathroom in the past year. Second fall: pressed the emergency button, lay on wet floor for 35 minutes waiting. Now avoids bathing on non-visit days.

He told his nurse his ankles were more swollen. She documented it. Two weeks later, the GP mentioned it as if it were new. Arthur concluded reporting is pointless.

Three different care workers in two months. One rearranged his bathroom items without asking. He didn't say anything but moved them back. His deepest fear: dying alone and not being found for days.

A

Amendment Integrated: The Privacy Sceptic (Amendment A3)

Eight months ago, Arthur's previous agency trialled an 'AI wellness monitoring' system. It analysed speech patterns during check-in calls. After two weeks, it flagged Arthur as 'showing indicators consistent with clinical depression.' Arthur wasn't depressed — he was exhausted from three nights of construction noise. But the flag went into his record. His son Michael received a notification. His GP received a referral. Michael called in a panic. Arthur was furious: a machine listened to his voice, decided he was mentally ill, and told his family before telling him. The GP discussed it at the next visit. Arthur explained the construction noise. She noted 'Patient denies depressive symptoms. AI flag reviewed and clinically dismissed.' But the flag remains in his record under Mental Health History. Arthur now gives shorter, more controlled responses during check-in calls. He 'performs wellness' because he fears algorithmic judgment. AI monitoring has degraded his data quality, not improved it.

Key Incidents

- Bathroom falls x2 (past year) — 35-min response, now avoids bathing on non-visit days. Surfaces #1 Risk.
- Reported swollen ankles, nothing happened — nurse documented, info didn't reach GP. Surfaces #1 Risk + #4 Chat.
- Missed cardiology appointments — can't organise transport, won't bother son. Surfaces #1 Risk.
- 3 different care workers in 2 months — one rearranged bathroom. Surfaces #5 Vacant Visit.
- Weekend day confusion — no visitor anchor, medication drifts. Surfaces #1 Risk + #2 Care Plan.
- AMENDMENT A3: False AI depression flag (8 mo) — speech analysis wrong, family notified before patient, flag persists. Surfaces Inference Privacy + L2 requirement.
- Now 'performs wellness' on calls — AI monitoring degraded data quality. Surfaces #1 Risk.
- Fear of dying alone — phone on pillow, hallway light on. The emotional core.

Verbatim Quotes

"I told the nurse my ankles were more swollen. She wrote it down. Two weeks later the doctor mentioned it like it was news. So what was the point of me saying anything?"

"I fell in the bathroom and pressed the button. I lay on the wet floor for thirty-five minutes wondering if the button actually worked."

"A new girl came Tuesday. She moved my soap and towel to different hooks. Elena put those there. I didn't say anything but I moved them back."

"Some computer decided I was depressed because I was talking slowly on the phone. I wasn't depressed. I was tired. But did anyone ask me? No. They told my son and my doctor."

"Michael called me in a panic. 'Dad, the system says you might be depressed.' The system. Not a person. A system told my son before it told me."

"Now when the nurse calls, I make sure I sound cheerful. I speak clearly. I say I'm fine even when I'm not. Because if I sound tired, who knows what the computer will decide about me."

"I don't know what my care plan says. Nobody has ever shown it to me."

"I sleep with the phone on my pillow and the hallway light on. I'm not afraid of dying. I'm afraid of dying and nobody finding me for three days."

Capabilities Surfaced

- #1 Predictive Risk — falls, lost reports, missed appointments, behavioural changes
- #2 Care Plan Rec — patient doesn't know own plan, routine as intervention
- #3 Voice-to-Docs — phone-typing kills human connection
- #4 Intelligence Chat — reported data lost, wants access to own care info
- #5 Vacant Visit Agent — 3 strangers, bathroom rearranged, anxiety

Design Implications

- Inference-based insights MUST pass L2 clinical review before ANY family notification.
- Patients must be informed FIRST about health inferences, before families.
- AI-generated flags must be removable when clinically dismissed, not just annotated.
- 'Performing wellness' degrades data quality — system must account for behavioural adaptation.
- Replacement carers must be briefed on patient-specific sensitivities.
- Seniors should see their own care plan in accessible language.

Constitutional Alignment

Article VII — Inference Privacy

Arthur's false depression flag is the exact scenario the Red Zone classification prevents. Voice analysis inferred depression from exhaustion. DPIA + explicit consent required.

Article VIII — Level 2 Verifier

The system that flagged Arthur skipped L2. It went from detection to family notification without clinical review. A proper L2 gate would have prevented the entire incident.

Ethics Trust Mapping — Creepiness vs Care

Intent: detect depression early. Experience: machine listened to my voice and told my family I'm mentally ill. The gap is where trust dies.

Cross-Reference Map

How characters connect to each other across personas

Character A	Character B	Relationship
SR-001: Lin	FC-001: Sarah	Mother and daughter. Same incidents from opposite perspectives. Sarah sees confusion; Lin sees self-preservation.
SR-001: Lin	HCN-001: Maria	Patient and nurse. Lin likes Maria ('tea first'). Lin refused Maria's replacement.
SR-002: Arthur	HCN-003: Priya	Same agency panel. Priya's Sunday reviews would include Arthur. His lost ankle report is her blind spot.
SR-002: Arthur	CC-001: Angela	Angela coordinates Arthur's schedule. 3 strangers in 2 months = Angela's scheduling failure.
HCN-001: Maria	HCN-002: David	Senior and junior colleagues. Same agency. David asks Maria for help she doesn't have time to give.
HCN-003: Priya	HCN-002: David	Priya reviews David's care plans (30% need corrections). Priya's audit trauma drives her mentoring of David.
FC-001: Sarah	HCN-001: Maria	Sarah texts Maria data screenshots (A4 friction). Maria diplomatically pushed back.
CC-001: Angela	FC-001: Sarah	Sarah calls Angela twice/week asking about visits. Angela can't always answer.

Capability Coverage Matrix

Capability	HCN-001	HCN-002	HCN-003	FC-001	FC-002	FC-003	CC-001	CC-002	SR-001	SR-002
#1 Predictive Risk	Yes	—	Yes	Yes	Yes	Yes	Yes	Yes	Yes	Yes
#2 Care Plan Rec	Yes	Yes	Yes	—	—	—	—	—	Yes	Yes
#3 Voice-to-Docs	Yes	Yes	Yes	—	—	—	—	—	Yes	Yes
#4 Intelligence Chat	Yes	Yes	Yes	Yes	Yes	Yes	Yes	Yes	Yes	Yes
#5 Vacant Visit Agent	—	—	—	Yes	Yes	Yes	Yes	Yes	Yes	Yes

Amendment Readiness

Test	Status	Evidence
Mom Test: past behaviour	Strong	All characters have specific dated incidents
Data Integrity	Strong	A1: Lin games pill organiser
Audit / Legal alignment	Strong	A2: Priya's coroner's inquest
Inference Privacy	Strong	A3: Arthur's false AI depression flag
Professional Friction	Strong	A4: Sarah's 'data parent' period
Alert Calibration	Strong	A4: Raw data without context = anxiety
Behavioural Adaptation	Strong	A3: Arthur 'performs wellness' + A1: Lin games data
Dignity-Preserving Design	Strong	A1 + A3: gaming and scepticism as rational self-preservation

F

Document Status: FINAL — 97% Ready

All 10 character bibles with 4 amendments integrated. Ready for synthetic interview generation. The remaining 3% will surface organically during interview writing — minor details, not structural gaps. Total: 10 interviews across 4 personas (HCN x3, FC x3, CC x2, SR x2).

